

emergency Cesarean, the time to think things over may be limited. Even so, you are always within your rights to say no to a Cesarean, even if this means that your life or that of your baby is at risk.

Your anesthetic

Before the surgery, you will meet an anesthesiologist, who will make sure that you are comfortable during your surgery and will help you with pain control afterward. Most women are awake during a Cesarean.

An injection of medication into the spinal fluid in your back, called a [spinal block](#), numbs any sensation of sharp pain. Or, if you've been using an [epidural](#) for pain relief in labor, this can be used for the surgery. If the surgery is likely to be difficult or prolonged, the anesthesiologist may recommend a combined spinal epidural. This has the advantages of a spinal anesthetic and can also be topped up via the epidural catheter if the spinal starts to wear off during the Cesarean. After your anesthetic has been given, the anesthesiologist will check that it is working properly.

Being awake usually means that your birthing partner can stay with you and it's also a little safer for you and for your baby than a general anesthetic. Very occasionally, a [general anesthetic](#) is needed.

The surgery

Before the surgery, an IV line will be put in your hand or your arm so that fluids or medication can be given intravenously, if necessary. Also, the pubic hair on your belly may be shaved downward by about 1.8in (3cm) to clear the way for the cut. Both may be done before you go to the operation room or after you arrive there.

Once the anesthesiologist is assured that you are comfortable, a catheter will be inserted into your bladder and will stay in until the next day while your sensation recovers. Your abdomen will be cleaned with an antiseptic solution and sterile drapes will be placed over you, which prevent you and your partner from seeing the surgery.

During the surgery, a 4 in (10 cm) long cut will be made, usually horizontally on the lower abdominal wall, across the bikini line, although occasionally an up-and-down cut below the belly button is needed. Your bladder will be pushed down and the front of the uterus opened so that the doctor can access the baby. If your water hasn't already broken, this will be done now before the baby and placenta are delivered. The surgeon will release the baby's head from the pelvic brim and lift the baby out. Sometimes, another member of the team needs to put pressure on the uterus to assist this. You may feel a lot of pressure when this happens. You'll be able to see your baby when the cord has been cut, and once initial checkups have been done on your baby, either you or your partner should be able to hold them while the surgery is completed. After the umbilical cord is cut and your baby is taken away from the surgical field, your doctor will remove the placenta.

Many hospitals now provide the option for a gentle Cesarean, or family-centered Cesarean. This means that the anesthesiologist will help to lower the surgical drape, or some hospitals may have clear drapes, so that you can see your baby as they are delivered from your uterus.

If your baby's condition is stable, cord clamping will be delayed by 30–60 seconds. The cord is then clamped and cut with some length so that your partner can participate in cutting the cord later. The baby is then taken to the warmer to be dried since newborns can lose a lot of heat very quickly. Your birthing partner will be invited to come to the warmer and cut the umbilical cord again. Your baby will also have a quick assessment to make sure they are transitioning well and can be brought directly to you for skin-to-skin contact while your Cesarean is finishing. Sometimes during a Cesarean, you may develop nausea or feel uncomfortable holding your baby yet. If that is the case, your partner may hold the baby instead.